Cubby Bed LMN guide for clinical providers

This guide provides information on the process of drafting a Letter of Medical Necessity (LMN) and the type of information that health insurance payers often require. Note that some payers will have specific forms that must be used to document an LMN or in addition to an LMN.

Below, you will find detailed descriptions of features that support safety and sensory needs. As you build your LMN, consider how each feature directly benefits this specific patient.

Answering each of the LMN Guide's questions/prompts in a full, patient-specific sentence will help document how each feature addresses the patient's sleep safety/sleep quality concern.
When detailing alternative solutions, be sure to include in your response why each was considered, tried and failed, or contraindicated.

Note: This guide is for informational purposes only and does not constitute legal, reimbursement or medical advice and does not guarantee insurance approval. You have the sole responsibility for determining medical necessity and complying with payer coverage policies. Coverage and reimbursement requirements are complex and frequently changing; you are encouraged to review the patient's health insurance plan and its policies for more information.

Checklist for an LMN

Each payer has specific coverage criteria, and the following is an overview of the common types of information required for an LMN.

- ☐ Clearly states the medical **diagnoses** that negatively affect the patient's sleep safety and quality and how they affect the patient's daily life
- ☐ Lists **equipment requested** and how the equipment will improve the patient's sleep safety and quality
- ☐ Describes the patient's **sleep situation, documented sleep behaviors, safety concerns, and any previous injuries**
- ☐ **Explains past failed interventions** individually and includes detailed reasons why they were ineffective
- ☐ Details how the Cubby Bed's **features specifically address the patient's sleep safety/sleep quality concerns and needs**
- ☐ Confirms that there is not a **medical need for head/foot articulation*** of the bed
- ☐ Discusses the **least-costly alternatives and relative patient benefit**
- ☐ Addresses and includes the **patient's specific policy coverage criteria**

***Important notice**
Head and Foot Articulation - Does the patient require head and foot articulation? The Cubby Bed does not currently offer head or foot articulation. If your patient medically requires head or foot articulation, then a different product intervention should be considered.

Table of contents

Tips for best use of this document: This document covers common medical conditions and corresponding Cubby Bed features. To efficiently locate patient-specific information, find the medical condition in the Table of Contents and use the quick links provided to navigate to and between relevant sections.

Addressing safety, sleep, and sensory concerns with Cubby Bed: features

Evaluate the patient based on safety and sleep concerns. Please see the following Cubby Bed features that address these concerns, as well as descriptions of how each feature addresses the specific medical/safety concern. Use only as a guideline on what is medically necessary for the patient.
Tip: Press “Crtl” + “F” to search this document

Elopement + wandering
Safety + sensory feature
Enclosed Tension-Padded Canopy with Safety Zipper Pockets: This creates an environment that reduces the risk of elopement/wandering. The durable, soft, and breathable canopy fabric provides tension padding, similar to a trampoline wall, intended to keep the user safe and secure in their Cubby Bed through all hours of the night. Safety zipper pockets decrease the likelihood of the user escaping their room, which can lead to safety concerns that have led to injury. The caregiver and the user are both able to operate the zipper should they choose to. **Dream Hub (Sensory + Monitoring Hub) with Safety Monitoring Camera + Mic:** The integrated Dream Hub provides high-resolution monitoring into daily activity through customizable sensors. This technology offers real-time notifications via a secure digital feed, allowing caregivers to stay informed without disrupting the user's environment. By documenting activity levels, the system provides supplemental information that can be shared with a care team. Additionally, the Dream Hub’s two-way audio helps caregivers maintain connection and provide remote reassurance when needed.
Documentation guidance
Enclosed Tension-Padded Canopy with Safety Zipper Pockets Please describe in detail how frequently the patient has attempted to leave their bed or bedroom unsupervised during the night and specify any instances where they have wandered into unsafe areas such as staircases, kitchens, bathrooms, or outside the home. Provide a comprehensive account of any injuries the patient has experienced due to wandering or elopement, including the dates, frequency, and details of each incident, as well as any ER visits or hospitalizations that resulted from these events (Attaching chart notes listing these events would be helpful for the submission). If the patient lives near a body of water or a busy street, please elaborate on any previous or potential safety concerns related to this proximity due to elopement/wandering behaviors. Please explain the extent of the patient's difficulty in understanding personal safety and recognizing dangerous situations, and list any previous or potential safety concerns related to this difficulty that are connected to elopement/wandering.

Dream Hub (Sensory + Monitoring Hub) with Safety Monitoring Camera + Mic
Please describe how real-time alerts for sound and movement would specifically enable caregivers to intervene promptly when the patient attempts to elope. Explain how the use of the Dream Hub would help with elopement risk reduction.
Describe any instances where the patient has left their bed unnoticed, including the specific consequences that occurred. Explain how a 2-way communication system would be used to verbally redirect the patient and de-escalate future elopement attempts.
Explain how recorded video evidence would be used to analyze the patient's wandering behavior, identify patterns, and develop more effective intervention strategies.

Common "alternatives" payers request to rule out first

Have past interventions been less effective in addressing elopement risks? List which ones and why they did not or would not address this patient's concerns. Common past interventions include: (explain which interventions were used for this specific patient):

- Door and window locks
- Baby monitor
- Bed alarms
- Baby gates
- Medication
- Mattress of the floor

Fall risk

Safety + sensory feature

Enclosed Tension-Padded Canopy: The 360-degree Canopy provides a secure sleeping environment for the patient, reducing the frequency of self-injurious behavior (SIB).
Durable Dual-Layer Mesh + Fabric Doors: The doors can be customized to meet the specific needs of the user, creating a comforting place to decrease sensory stimulation with the option of mesh doors for daytime visibility and fabric doors for nighttime enclosure.
Safety Zipper Pockets: A safety measure to decrease the likelihood of the user escaping their room, which can lead to safety concerns that have led to injury.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: Alerts the caregiver of typical sounds and movements to enable them to immediately detect restlessness, unsafe positioning, or attempts to climb out of bed, all of which can lead to falls. This allows caregivers and the medical team to analyze patterns and implement strategies to reduce the frequency of self-injurious behavior (SIB). The 2-way communication system with a speaker and mic allows caregivers to gently redirect the patient, provide reassurance, and intervene verbally if they notice unsafe movements.

Documentation guidance

Enclosed Tension-Padded Canopy and Durable Dual-Layer Mesh + Fabric Doors: Describe each instance where the patient has fallen out of bed, including how often a night, the specific circumstances surrounding the fall, the frequency of these incidents over the past month/year, and a detailed description of any resulting injuries, emergency department visits, or hospitalizations. Please also include any medical reports or documentation that supports this account.

Explain the patient's difficulties with balance, coordination, or mobility, detailing how these challenges affect their risk of falling and how the Cubby Bed would address this specific concern. Provide details about whether the patient moves excessively during sleep, leading to an increased likelihood of rolling out of bed.

Safety Zipper Pockets: Please describe in detail any instances where the patient has attempted to leave their bed during the night, including the frequency and timing of these attempts, the methods used by the patient to try to exit the bed, and the specific ways in which these attempts have increased the risk of falls.

Please explain the extent to which the patient demonstrates a lack of awareness of their surroundings, and describe how this lack of awareness contributes to their tendency for nighttime wandering. Please provide a brief description of the emergency plan, detailing how the patient will exit the room or house in the event of a fire or other emergency. Please also include a comparison of the emergency plan to any alternative solutions.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: Describe the level of assistance and support the patient currently requires to minimize the risk of falling, including the frequency and intensity of monitoring. Explain how remote monitoring will aid in supervision.

Provide a comprehensive account of any past incidents where a fall or unsafe movement was discovered only after it had already occurred, including the details of each incident and the resulting consequences. Explain how the Dream Hub's real-time alerts for movement and sound would enable caregivers to intervene proactively and how it has the potential to prevent similar incidents from happening in the future.

Describe how the availability of recorded footage would allow caregivers and the care team to analyze the patient's behavior, identify patterns that contribute to falls, and develop more effective preventative strategies. Additionally, explain in detail how the 2-way communication system would be utilized to verbally redirect and calm the patient before they attempt to exit the bed.

Common "alternatives" payers request to rule out first
Have previous strategies not worked to address concerns related to reducing the frequency of self-injurious behavior (SIB)? Please detail which ones and why they did not or would not address this patient's concerns. ● Alternative safety beds or bed tents ● Bed rails ● Helmet

Burrowing + entrapment
Safety + sensory feature
Safety Sheets: The Safety Sheets are made of a soft and breathable microfiber, which are zipped and secured to the interior canopy walls of the bed, above the mattress, designed to create one even surface. This allows for the user's movement and is intended to guard against entanglement in loose sheets or entrapment within the space between the mattress and the edge of the bed. **Enclosed Tension-Padded Canopy:** The Enclosed Canopy and Safety Sheet system eliminates the need for rails by providing a safe and secure sleeping environment for the user and are designed to minimize the risk of entrapment in the 7 zones identified by the FDA in a traditional hospital bed. **Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic:** The integrated Dream Hub provides high-resolution monitoring into daily activity through customizable sensors. This technology offers real-time notifications via a secure digital feed, allowing caregivers to stay informed without disrupting the user's environment. By documenting activity levels, the system provides supplemental information that can be shared with a care team. Additionally, the Dream Hub's two-way audio helps caregivers maintain connection and provide remote reassurance when needed.
Documentation guidance
Safety Sheets: Please describe any instances where the patient has become entangled in loose bedding, including the resulting distress or safety risks. Describe any attempts the patient makes to burrow under blankets or mattresses, explaining how this creates a suffocation or entrapment risk. Also, describe any occasions when the patient has been found wedged between the mattress and the bed frame, emphasizing the associated safety hazards. Explain how the patient's sensory-seeking behaviors manifest, specifically how they cause the patient to dig into bedding or desire to be in a tight space.

Describe in detail any past interventions such as traditional beds, cribs, a mattress on the floor, or baby monitors that were implemented to aid in the prevention of entrapment or burrowing, and explain why each of these interventions provided limited solutions. In what ways would the Safety Sheets assist in minimizing the occurrence of these issues? **Enclosed Tension-Padded Canopy:** Please describe any instances where the patient has fallen between bed rails or become entrapped in gaps within a traditional bed. Explain how the patient pushes against hard surfaces during sleep, detailing how this increases the risk of bruising or injury. Discuss how a soft yet secure enclosure with hypoallergenic, gentle fabrics, insulated walls for noise reduction, dim lighting options, and potentially calming weighted features would address anxiety or sensory-seeking behaviors that contribute to burrowing. **Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic:** Please describe in detail any instances where the patient has been discovered in an unsafe position, such as being stuck or struggling, and explain the specific circumstances, the length of time they were in that position unnoticed, and any resulting consequences or injuries. Explain how real-time video monitoring and motion alerts would enable caregivers to identify and respond to potential entrapment risks before they become dangerous. Additionally, describe how a recorded history of the patient's nighttime movements would provide valuable information about the frequency and underlying reasons for their burrowing or entrapment behaviors, and how this information would be utilized to improve care. Explain precisely how the 2-way communication system would be used to verbally redirect the patient away from unsafe burrowing behaviors without the caregiver needing to physically enter the room.
Common “alternatives” payers request to rule out first
How did past interventions provide limited support for burrowing and entrapment risks? Please list which ones and why they did not or would not address this patient’s concerns. ● Traditional beds ● Cribs ● Mattress on the floor ● Baby monitors

Self-injurious, aggressive, and destructive behaviors

Safety + sensory feature

Enclosed Tension-Padded Canopy: The Tension Padding is unique to the Cubby Bed as the canopy is tautly stretched across the custom steel frame. The fully enclosed, 360-degree canopy supports independent movement and helps protect the patient from self-harm caused by contact with hard surfaces resulting from self-injurious, aggressive, destructive, and/or impulsive unregulated behaviors. The canopy is tensioned enough to always stay 4-6” away from any hard parts of the steel frame.

Durable Dual-Layer Mesh + Fabric Doors: The canopy has 3 entrances into the bed, equipped with heavy-duty dual-layered mesh + fabric doors, secured with premium zippers. The inclusion of zippered doors, operable from both the interior and exterior, is intended to help reduce potential self-injurious, aggressive, and destructive behaviors within the bedroom. This feature addresses concerns that might arise if doors were absent in such situations.

Full-Sized Bed Steel Frame: The canopy has 3 entrances into the bed, equipped with heavy-duty dual-layered mesh + fabric doors, secured with premium zippers. The steel frame can hold up to 1200 lbs.

Documentation guidance

Enclosed Tension-Padded Canopy: Describe any self-injurious behaviors the patient has exhibited during sleep, such as head-banging, hitting, or other aggressive or impulsive actions. Include details on how frequently these behaviors occur, the severity of the impact, and any injuries that have resulted. Explain whether these behaviors increase during specific situations, such as changes in routine, illness, or sensory overload.

Explain the level of supervision required and how the required level of nighttime supervision is related to reducing the frequency of self-injurious behavior (SIB)/helping ensure the patient's safety. Describe whether a caregiver must remain awake, monitor the patient with cameras, or frequently intervene to help prevent injury. Include any instances where lack of supervision has resulted in harm, distress, or unsafe situations.

Discuss how a 360-degree tensioned canopy would impact the patient’s ability to move freely while helping to prevent injury. Explain how the design of the canopy would allow for unrestricted movement while reducing the chances of the patient hitting hard surfaces, falling out of bed, or becoming entrapped.

Durable Dual-Layer Mesh + Fabric Doors
Provide details on any instances where the patient has hit, kicked, or thrown objects due to

aggression or frustration. Describe how often these behaviors occur and what triggers them. Explain whether these actions have caused damage to the sleep environment, injury to the patient or others, or disruption to sleep patterns.

Describe how a secure yet breathable enclosure could help manage the patient's sensory needs, reduce overstimulation, and promote a calming sleep environment. Explain whether the patient has a history of sensory processing challenges, hypersensitivity to external stimuli, or difficulty settling at night due to an overactive nervous system. Include details on whether the patient responds positively to enclosed spaces, compression, or reduced visual/auditory input.

Full-Sized Bed Steel Frame

Explain whether the patient has damaged, broken, or outgrown previous beds due to aggressive movements or forceful impact, and how this has affected their sleep and safety. Provide examples of beds, frames, or mattresses that have been used in the past and whether they were effective. Describe any instances where beds were replaced due to excessive wear, breakage, or safety concerns.

Describe the patient's need for a larger sleeping space and how it would help manage self-injurious, aggressive, or destructive behaviors during sleep. Explain whether the patient moves forcefully during the night, engages in behaviors such as head-banging, punching, or kicking, or has a history of breaking bed frames, damaging mattresses, or injuring themselves due to restricted space. Discuss how a larger, reinforced sleep environment could provide the necessary room for movement.

Common "alternatives" payers request to rule out first

How have past interventions offered limited assistance in addressing self-injurious, aggressive, or destructive behavior? Please list which ones and why they did not or would not address this patient's concerns.

- Traditional beds
- Mattress on the floor
- Baby monitors
- Helmet

No recognition of danger

Safety + sensory feature

Enclosed Tension-Padded Canopy: The 360-degree enclosed canopy offers a secure sleeping space, which may help to reduce the frequency of self-injurious behavior (SIB). It also provides a contained sleeping space, reducing the risk of wandering, which could put the user with no recognition of danger at risk for getting into unsafe situations unsupervised at night outside of their bedroom.

Safety Zipper Pockets: The Canopy is equipped with interior safety pockets, which hide the interior zippers of the doors from the user of the bed. This helps secure the user of the bed inside the bed, reducing the risk of them being unsupervised and getting into situations they might not know are dangerous.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: The HD video and night vision camera with multi-device viewing capability will allow caregivers to observe behavioral patterns and promote early intervention and management of potential dangers and risks.

Documentation guidance

Enclosed Tension Canopy: Describe any instances where the patient has fallen out of bed during sleep or left their bedroom unsupervised. Provide details on how often this occurs, any resulting injuries, and the measures that have been taken to help prevent these incidents. Explain whether the patient has difficulty understanding boundaries, experiences nighttime disorientation, or engages in wandering behaviors that pose a safety risk.

Explain how a fully enclosed, sensory-friendly sleeping environment would help reduce the risk of wandering and keep the patient safe at night. Discuss how an enclosed space could provide a secure yet calming sleep setting, reduce sensory overstimulation, and potentially minimize disruptions from external stimuli.

To support elopement risk reduction. Include any previous attempts to secure the patient's sleep environment and their effectiveness. List any previous attempts to secure the patient's sleep environment and their effectiveness.

Safety Zipper Pockets: Describe the additional security measures the patient requires to reduce the risk of elopement or wandering. Explain whether the patient has a history of unlocking doors, leaving the home unsupervised, or requiring locked environments to ensure safety. Provide details on any past incidents where the patient's wandering has led to dangerous situations and how these have impacted caregivers.

Discuss how the patient's inability to recognize danger has led to safety concerns in the past. Provide examples of situations where the patient has been at risk due to a lack of awareness of hazards, such as leaving the home, interacting with unsafe objects, or engaging in dangerous behaviors. Explain how these incidents were managed and whether additional safety measures were needed.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: Explain how remote monitoring of the patient would help caregivers be alerted to early signs of distress, agitation, or unsafe behaviors. Describe the types of behaviors that signal potential danger and how real-time monitoring could assist caregivers in responding quickly. Discuss whether the patient is prone to sudden emotional or behavioral escalations that require immediate intervention.

Describe how recording behavioral patterns could help caregivers and the care team recognize early warning signs and intervene before a medical emergency occurs. Explain how tracking changes in movement, vocalizations, or sleep disturbances could support medical or behavioral interventions. Provide examples of how past observations have helped in managing the patient's condition.

Common "alternatives" payers request to rule out first

Have previous strategies provided limited success in managing potentially dangerous situations for the patient? Please list which ones and why they did not or would not address this patient's concerns.

- Behavioral therapy
- Baby monitors
- Padded enclosed bed tents
- Helmet
- Padding on the wall

Seizure activity

Safety + sensory feature

Enclosed Tension-Padded Canopy: The enclosed canopy with tensioned padding can offer a secure sleeping environment for users who experience sudden and unpredictable movements during seizures. In some cases, users with seizures may injure themselves by hitting their head or other parts of the body against a standard bed or surrounding objects. The enclosed canopy provides a sensory-friendly environment to promote relaxation and better sleep. The enclosed canopy also helps block out external lights, muffle sounds, and other environmental factors that may trigger seizures.

Safety Sheets: The Safety Sheets zip and lock to the interior side walls of the bed, above the mattress, reducing the risk of entrapment between the mattress and the side of the bed if they have a seizure and cannot control their movements.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: The Dream Hub safety monitoring camera provides HD video monitoring with night vision and can record, which can provide videos to send to the user's medical teams to examine sleep and seizure activity. The camera is connected to an app on the caregiver's phone for easy access to check-in at any time and can be set up to send alert notifications for sound and movement. The alerts can improve the response time to administer any medication.

Dream Hub (Sensory + Monitoring Hub): Light + Speaker: The light and speaker system can provide a calming and relaxing environment for the user of the bed to reduce stimulation.

Documentation guidance

Enclosed Tension-Padded Canopy: Describe the patient's history of sudden and unpredictable seizures, including frequency and type. Explain how these seizures impact their daily life, sleep patterns, and overall well-being. Provide details on whether specific triggers, such as overstimulation or disrupted sleep, contribute to seizure activity.

Explain whether the patient has ever hit their head or limbs on a standard bed frame or nearby objects during a seizure. Describe any resulting injuries, the severity of impacts, and any prior safety measures that were implemented in an effort to reduce harm. Discuss whether traditional bed setups have been inadequate in protecting the patient during seizures.

Discuss how a dark, quiet, and enclosed sleep environment could help regulate the patient's sensory system and improve sleep quality, and how that may impact seizure activity.

Explain whether sensory overstimulation contributes to the patient's seizures and how creating a calming environment may help with relaxation and seizure management.

Safety Sheets: Provide details on any incidents where the patient has become trapped between the mattress and bed frame during a seizure. Explain how this has posed a safety risk and whether alternative bedding solutions have been attempted. Discuss the potential benefits of using Safety Sheets to minimize the risk of entrapment.

Describe whether the patient experiences involuntary movements that could lead to entanglement in loose bedding. Explain how these movements have affected the patient's safety during sleep and whether modifications to the sleep setup have been necessary to reduce risks.

Explain whether the patient has required physical repositioning during or after a seizure. Describe the caregiver's role in ensuring the patient's safety post-seizure and whether a specialized bed setup could aid in managing these needs more effectively.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: Discuss how real-time video monitoring and alerts would enable caregivers to more rapidly identify and react to seizure activity. Provide examples of past situations where seizures went unnoticed or required urgent intervention. Explain whether emergency medication or repositioning is often necessary and how monitoring could support timely responses.

Describe any past instances where the patient had seizures that were only discovered after they occurred, increasing the medical complications due to delayed seizure intervention. Explain whether baby monitors or other basic monitoring tools have been used and why they may have been insufficient in detecting and alerting caregivers to seizure activity.

Explain how recorded footage of seizures could help the medical team analyze patterns and adjust treatment. Discuss how tracking seizure frequency, duration, and potential triggers through video monitoring could provide valuable data for treatment modifications and improved management strategies.

Dream Hub (Sensory + Monitoring Hub): Light + Speaker: Explain whether the patient has difficulty falling or staying asleep and how a consistent sleep environment could positively impact their neurological health.

Explain whether gentle light transitions and soothing sounds would help create a relaxing sleep environment that minimizes sensory overstimulation. Discuss whether the patient is sensitive to sudden changes in light, sound, or environment and how structured sensory regulation could improve sleep quality.

Provide details on any correlation between the patient's disrupted or poor-quality sleep and increased seizure activity. Explain whether sleep deprivation or fragmented sleep has led to a noticeable increase in seizures and how improving sleep conditions could help manage seizure frequency and severity.

Common "alternatives" payers request to rule out first

Did previous interventions provide limited success in managing the frequency or severity of seizures? Please list which ones and why they did not or would not address this patient's concerns.

- Medication
- Baby monitors

PICA

Safety + sensory feature
Enclosed Tension-Padded Canopy/Safety Zipper Pockets/ Safety Sheets: The combination of the enclosed canopy, safety zipper pockets, and Safety Sheets provides a safe and secure sleeping environment with walls and a roof that can limit access to non-food items. The enclosed environment may help deter the user of the bed from attempting to ingest non-food items during the night, making it more difficult to even access non-food items. The enclosed tension-padded canopy has no foam padding, so there is no risk of the user of the bed chewing on foam padding, which can be dangerous, toxic, and cause damage to the bed. **Full-Sized Bed Steel Frame:** The canopy is stretched around a steel frame on the bed. The tension of the canopy around the frame provides a barrier between the user and the bed frame, and by having a steel frame, it does not allow the user of the bed to chew on the frame while secured inside of the bed like they might with a wood frame. **Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic:** The Safety monitoring camera provides remote access for the caregiver to supervise and get notified through alerts directly to their phone for sounds and motion to check in on the user of the bed, making sure they are not attempting to ingest non-food items.
Documentation guidance
Enclosed Tension-Padded Canopy, Safety Zipper Pockets, and Safety Sheets: Describe the patient's history of attempting to ingest non-food items during sleep or nighttime hours. Include specific examples of items the patient has tried to ingest, how often these behaviors occur, and any trends related to sensory-seeking behaviors, anxiety, or nighttime restlessness. Explain whether the patient has ever required medical intervention due to ingesting hazardous items. Provide details on any past medical emergencies, such as choking incidents, gastrointestinal blockages, or poisoning, and describe how these events were managed. Discuss whether the patient has demonstrated the ability to remove or damage traditional safety interventions, such as bed rails, mesh covers, or enclosed tents. Explain any previous attempts to secure the sleep environment and why these measures were ineffective in preventing access to non-food items. **Full-Sized Bed Steel Frame:** Describe any behaviors the patient has exhibited related to chewing on furniture, bedding, foam, fabric, or bed frames. Provide details on whether these behaviors are persistent, destructive, or linked to sensory-seeking tendencies. Explain any safety concerns related to material ingestion and the durability of the sleep environment.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: Explain whether continuous or frequent monitoring is needed to ensure the patient does not ingest non-food items. Describe the level of supervision required at night, how often caregivers need to intervene, and any tools currently used to monitor the patient's safety. Describe any instances where the patient required emergency intervention due to the unsupervised ingestion of non-food items. Provide examples of situations where ingestion led to choking, medical treatment, or hospitalization and how these events could have been prevented. Discuss whether the patient has previously used a baby monitor and attempted to tamper with or chew on the cord. Explain whether the patient has a history of damaging monitoring devices, accessing unsafe objects despite supervision, or requiring alternative monitoring solutions to ensure safety.
Common "alternatives" payers request to rule out first
Were previous interventions unsuccessful in addressing PICA? Please list which ones and why they did not or would not address this patient's concerns. ● Other safety beds or bed tents ● Mesh covers ● Baby monitors

Stimming behavior
Safety + sensory feature
Enclosed Tension-Padded Canopy: The enclosed tension-padded canopy may provide an added layer of safety depending on the type of stimming behaviors a user exhibits. If a user exhibits self-injurious behaviors, the enclosed tension-padded canopy can help protect against head-banging and other self-harm behaviors during sleep. The enclosed canopy can provide an environment that can help the user of the bed self-regulate. **Durable Dual-Layer Mesh + Fabric Doors:** The durable dual-layer mesh + fabric doors can provide sensory benefits depending on the type of stimming behaviors a user exhibits and allow visual input while ensuring safety.
Documentation guidance
Enclosed Tension-Padded Canopy: Describe any self-injurious stimming behaviors the patient

exhibits during sleep, such as head-banging or repetitive body movements. Include details on the frequency, intensity, and potential triggers of these behaviors, as well as any patterns observed over time. Explain whether these behaviors escalate in response to sensory overload, anxiety, or difficulty transitioning to sleep.

Explain whether the patient has ever required medical attention due to self-harm behaviors occurring at night or in an unsecured sleeping environment. Provide details on any past injuries, such as bruises, cuts, or concussions, and describe any medical interventions that have been necessary to address these concerns.

Discuss the patient's ability to self-regulate during sleep or upon waking in the middle of the night. Explain whether they struggle with calming themselves, engage in excessive movement, or require caregiver intervention to settle back to sleep. Include details on any current strategies being used and their effectiveness.

Describe any difficulties the patient has sleeping in an open environment and whether they require a more enclosed space for comfort and security. Explain whether an enclosed sleep environment has previously been attempted and how it impacted their ability to settle and remain asleep.

Durable Dual-Layer Mesh + Fabric Doors: Explain whether the patient benefits from customizable sensory input to help manage stimming behaviors, such as light coming through mesh versus a fully dark enclosure with fabric. Describe how different sensory conditions affect the patient's sleep quality, anxiety levels, and nighttime movements.

Discuss whether the patient has demonstrated a need for a darker, enclosed sleep space to feel secure and reduce overstimulation. Provide specific examples of behaviors that indicate distress in an open sleeping environment and how an enclosed space could provide a calming effect.

Explain how limiting external stimuli with fabric doors could help reduce nighttime awakenings and promote deeper sleep. Include details on whether light, noise, or other environmental factors frequently disrupt the patient's sleep and how creating a controlled environment could improve restfulness.

Dream Hub (Sensory + Monitoring Hub) Light with Speaker: Describe any struggle the patient has with falling asleep or maintaining a consistent sleep schedule due to sensory needs or stimming behaviors. Explain whether customizable lighting could help regulate their disrupted sleep-wake cycle and support a more structured nighttime routine.

Discuss whether the patient exhibits mood swings, anxiety, or emotional dysregulation that could benefit from customizable lighting and calming sound therapy. Provide examples of how sensory-based interventions have helped the patient in other contexts and whether a structured bedtime routine with controlled lighting and sound could improve emotional stability.

Explain whether guided meditative breathing programs could help the patient transition to sleep more easily and reduce bedtime anxiety. Describe any past efforts to introduce relaxation techniques and whether structured programs for breathwork or sound therapy could provide additional support in reducing nighttime distress.

Common "alternatives" payers request to rule out first

Have past interventions provided limited success in addressing stimming behavior? Please list which ones and why they did not fully address this patient's concerns.

- Behavior therapy, occupational/physical therapy
- Medication
- Sound machines
- Sensory and soothing bedtime routines

Sensory processing, hyper/hyposensitivity, overstimulation, and anxiety

Safety + sensory feature

Enclosed Tension-Padded Canopy: The enclosed canopy is designed with soft fabrics, calming colors, and the option to customize the environment using either the mesh doors or full fabric doors, which can promote sensory regulation to help the user feel more comfortable and less anxious in their sleep area. The canopy can also reduce external distractions that may interfere with sleep, such as light or noise, which can help users with overstimulation or anxiety who may be easily distracted or have difficulty falling asleep. The enclosed bed can provide a private and calming space where the user can engage in self-soothing activities like mindfulness exercises.

Durable Dual-Layer Mesh + Fabric Doors: The dual-layer doors can provide customization by using either the mesh doors or full fabric doors, which can promote sensory regulation to help the user feel more comfortable and less anxious in their sleep area.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: The safety monitoring camera can be used for the caregiver to check in at any time the user is in bed to make sure they are relaxed and not overstimulated. The caregivers can set up notification alerts on their phones for motion and sound. If the user is anxious or overstimulated, the caregiver can communicate via audio through the camera mic and speaker to provide verbal cues of relaxation, read bedtime stories, sing songs, or just affirm everything is okay.

Dream Hub (Sensory v Monitoring Hub): Light w/ Speaker: The customizable light allows the care giver to set up time intervals of the light while playing a variety of calming sounds , which can promote relaxation/sleep and reduce stress and anxiety.

Documentation guidance

Enclosed Tension-Padded Canopy: Describe how the patient experiences sensory sensitivities to light, sound, or movement and how these sensitivities interfere with their ability to fall or stay asleep. Provide details on specific sensory triggers that cause discomfort, distress, or overstimulation at bedtime. Explain how these sensitivities impact sleep quality and whether they lead to delayed sleep onset or frequent nighttime awakenings.

Explain whether the patient has difficulty filtering out external distractions and how this affects their ability to settle at night. Discuss whether background noise, changes in lighting, or environmental stimuli cause difficulty in transitioning to sleep or contribute to frequent night wakings.

Durable Dual-Layer Mesh + Fabric Doors: Describe how the ability to adjust sensory input, such as light filtering through mesh versus a fully enclosed fabric setup, impacts the patient's comfort and anxiety at bedtime. Provide details on whether having control over their sleep environment has been beneficial in reducing stress and promoting relaxation.

Discuss whether reducing visual and environmental stimuli would help prevent overstimulation and bedtime resistance. Explain how limiting distractions in the sleep space could create a calming environment, support self-regulation, and improve sleep consistency.

Explain whether the patient has previously experienced distress or difficulty sleeping in a fully open or traditional bed setup. Provide details on any past attempts to modify their sleep environment, how they responded, and whether a more enclosed, controlled space could improve their sleep experience.

Dream Hub (Sensory + Monitoring Hub): Safety Monitoring Camera + Mic: Describe whether the patient requires frequent reassurance or caregiver interaction to manage nighttime anxiety or overstimulation. Explain the level of caregiver involvement needed at night, including verbal reassurance, physical presence, or sensory accommodations.

Discuss whether the ability of a caregiver to provide real-time verbal cues or soothing reassurance via two-way audio would help the patient self-regulate. Explain how access to calming communication could reduce anxiety, help prevent distress, and help the patient return to sleep independently.

Provide details on any instances where the patient has experienced nighttime distress, meltdowns, or panic attacks that required immediate caregiver intervention. Describe the triggers, the severity

of these episodes, and the strategies used to help the patient recover. Explain how motion and sound alerts could help caregivers intervene before the patient escalates into a state of extreme anxiety or overstimulation. Discuss how real-time monitoring could reduce the frequency of full-blown meltdowns and allow for earlier, more effective intervention. Describe whether the patient has required the physical presence of a caregiver during bedtime and whether remote check-ins could promote greater independence. Explain whether transitioning to a monitored but more independent sleep setup could improve the patient's self-soothing abilities while still ensuring their safety. **Dream Hub (Sensory + Monitoring Hub): Light with Speaker:** Discuss whether the patient struggles with falling asleep or maintaining a consistent sleep-wake cycle due to sensory dysregulation. Provide details on how sensory needs impact their circadian rhythm and whether structured interventions have been attempted. Describe whether the patient has previously attempted sound machines, medication, or structured bedtime routines without experiencing significant improvement. Provide insight into which interventions have been tried, their effectiveness, and whether additional sensory accommodations are necessary for better sleep regulation.
Common "alternatives" payers request to rule out first
Have past interventions provided limited success in managing anxiety, hyper/hypo-sensitivity, overstimulation, and anxiety? Please list which ones and why they did not fully address this patient's concerns. ● Sound machines ● Sensory and soothing bedtime routines